





| Interaction table                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |  | System Organ Class                   |  | Adverse Reactions                                                                                                                                                                                                                                                                                            |  |
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| Interactions between voriconazole and other medicinal products are listed in the table below (once daily as "QD", twice daily as "BID", three times daily as "TID" and not determined as "ND"). The direction of the arrow for each pharmacokinetic parameter is based on the 90% confidence interval of the geometric mean ratio being within (←→), below (↓) or above (↑) the 80- 125% range. The asterisk (*) indicates a two-way interaction. AUC <sub>0-12</sub> , AUC <sub>0-24</sub> and AUC <sub>0-∞</sub> → represent area under the curve over a dosing interval, from time zero to the time with detectable measurement and from time zero to infinity, respectively. |  | Infections and infestations          |  | sinusitis, pseudomembranous colitis                                                                                                                                                                                                                                                                          |  |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |  | Blood and lymphatic system disorders |  | agranulocytosis <sup>1</sup> , pancytopenia, thrombocytopenia <sup>2</sup> , leukopenia, anaemia, bone marrow failure, lymphadenopathy, eosinophilia, disseminated intravascular coagulation                                                                                                                 |  |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |  | Immune system disorders              |  | hypersensitivity, anaphylactoid reaction                                                                                                                                                                                                                                                                     |  |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |  | Endocrine disorders                  |  | adrenal insufficiency, hypothyroidism, hyperthyroidism                                                                                                                                                                                                                                                       |  |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |  | Metabolism and nutrition disorders   |  | oedema peripheral, hypoglycaemia, hypokalaemia, hyponatraemia                                                                                                                                                                                                                                                |  |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |  | Psychiatric disorders                |  | depression, hallucination, anxiety, insomnia, agitation, confusional state                                                                                                                                                                                                                                   |  |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |  | Nervous system disorders             |  | headache, convulsion, syncope, tremor, hypertonia <sup>3</sup> , paraesthesia, somnolence, dizziness, brain oedema, encephalopathy <sup>4</sup> , extrapyramidal disorder <sup>5</sup> , neuropathy peripheral, ataxia, hypoaesthesia, dysgeusia, hepatic encephalopathy, Guillain-Barre syndrome, nystagmus |  |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |  | Eye disorders                        |  | visual impairment <sup>6</sup> , retinal haemorrhage, papilloedema, oculogyric crisis, diplopia, scleritis, blepharitis, optic atrophy, corneal opacity                                                                                                                                                      |  |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |  | Ear and labyrinth disorders          |  | hyperacusis, vertigo, tinnitus                                                                                                                                                                                                                                                                               |  |

| Medicinal product [Mechanism of interaction]                                                                                                                                                             |  | Recommendations concerning co-administration                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |  |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--|
| Astemizole, cisapride, pimozide, quinidine and terfenadine [CYP3A4 substrates]                                                                                                                           |  | Contraindicated                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |  |
| Carbamazepine and long-acting barbiturates (e.g., phenobarbital, mephobarbital) [potent CYP450 inducers]                                                                                                 |  | Contraindicated                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |  |
| Efavirenz (a non-nucleoside reverse transcriptase inhibitor) [CYP450 inducer; CYP3A4 inhibitor and substrate]                                                                                            |  | Use of standard doses of voriconazole with efavirenz doses of 400 mg QD or higher is <b>contraindicated</b> . Voriconazole may be coadministered with efavirenz if the voriconazole maintenance dose is increased to 400mg BID and the efavirenz dose is decreased to 300 mg QD.<br>When voriconazole treatment is stopped, the initial dose of efavirenz should be restored.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |  |
| Ergot alkaloids (e.g., ergolamine and dihydroergotamine) [CYP3A4 substrates]                                                                                                                             |  | Contraindicated                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |  |
| Rifabutin [potent CYP450 inducer] 300 mg QD<br>300 mg QD (co-administered with voriconazole 350 mg BID)*                                                                                                 |  | Concomitant use of voriconazole and rifabutin should be avoided unless the benefit outweighs the risk. The maintenance dose of voriconazole may be increased to 5 mg/kg intravenously BID or from 200 mg to 350 mg orally BID (100 mg to 200 mg orally BID in patients less than 40 kg). Careful monitoring of full blood counts and adverse                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |  |
| 300 mg QD (co-administered with voriconazole 400 mgBID)*                                                                                                                                                 |  | reactions to rifabutin (e.g., uveitis) is recommended when rifabutin is coadministered with voriconazole.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |  |
| Rifampicin (600 mg QD) [potent CYP450 inducer]                                                                                                                                                           |  | Contraindicated                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |  |
| Ritonavir (protease inhibitor) [potent CYP450 inducer; CYP3A4 inhibitor and substrate]                                                                                                                   |  | Co-administration of voriconazole and high doses of ritonavir (400 mg and above BID) is <b>contraindicated</b> . Co-administration of voriconazole and low dose ritonavir (100 mg BID) should be avoided, unless an assessment of the benefit/risk to the patient justifies the use of voriconazole.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |  |
| High dose (400 mg BID)<br>Low dose (100 mg BID)*                                                                                                                                                         |  | Contraindicated.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |  |
| St John's Wort [CYP450 inducer; P-gp inducer] 300 mg TID (co-administered with voriconazole 400 mg single dose)                                                                                          |  | Contraindicated.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |  |
| Everolimus [CYP3A4 substrate, P-gP substrate]                                                                                                                                                            |  | Co-administration of voriconazole with everolimus is not recommended because voriconazole is expected to significantly increase everolimus concentrations.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |  |
| Fluconazole (200 mg QD) [CYP2C9, CYP2C19 and CYP3A4 inhibitor]                                                                                                                                           |  | The reduced dose and/or frequency of voriconazole and fluconazole that would eliminate this effect have not been established. Monitoring for voriconazole-associated adverse reactions is recommended if voriconazole is used sequentially after fluconazole.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |  |
| Phenytoin [CYP2C9 substrate and potent CYP450 inducer] 300 mg QD<br>300 mg QD (co-administered with voriconazole 400 mg BID)*                                                                            |  | Concomitant use of voriconazole and phenytoin should be avoided unless the benefit outweighs the risk. Careful monitoring of phenytoin plasma levels is recommended.<br>Phenytoin may be co-administered with voriconazole if the maintenance dose of voriconazole is increased to 5 mg/kg IV BID or from 200 mg to 400 mg oral BID, (100 mg to 200 mg oral BID in patients less than 40 kg).                                                                                                                                                                                                                                                                                                                                                                                                                                                                |  |
| Anticoagulants                                                                                                                                                                                           |  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |  |
| Warfarin (30 mg single dose, co-administered with 300 mg BID voriconazole) [CYP2C9 substrate]<br>Other oral coumarins (e.g., phenprocoumon, acenocoumarol) [CYP2C9 and CYP3A4 substrates]                |  | Close monitoring of prothrombin time or other suitable anticoagulation tests is recommended and the dose of anticoagulants should be adjusted accordingly.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |  |
| Benzodiazepines (e.g., midazolam, triazolam, alprazolam) [CYP3A4 substrates]                                                                                                                             |  | Dose reduction of benzodiazepines should be considered.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |  |
| Immuno-suppressants [CYP3A4 substrates]<br>Sirolimus (2 mg single dose)<br>Ciclosporin (In stable renal transplant recipients receivingchronic ciclosporin therapy)<br>Tacrolimus (0.1 mg/kg singledose) |  | Co-administration of voriconazole and sirolimus is <b>contraindicated</b> .<br>When initiating voriconazole in patients already on ciclosporin it is recommended that the ciclosporin dose be halved and ciclosporin level carefully monitored. Increased ciclosporin levels have been associated with nephrotoxicity. <u>When voriconazole is discontinued, ciclosporin levels must be carefully monitored and the dose increased as necessary.</u><br>When initiating voriconazole in patients already on tacrolimus, it is recommended that the tacrolimus dose be dereduced to a third of the original dose and tacrolimus level carefully monitored. Increased tacrolimus levels have been associated with nephrotoxicity. <u>When voriconazole is discontinued, tacrolimus levels must be carefully monitored and the dose increased as necessary.</u> |  |
| Long Acting Opiates [CYP3A4 substrates] Oxycodone (10 mg single dose)                                                                                                                                    |  | Dose reduction in oxycodone and other long-acting opiates metabolized by CYP3A4 (e.g., hydrocodone) should be considered. Frequent monitoring for opiate associated adverse reactions may be necessary.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |  |
| Methadone (32-100 mg QD) [CYP3A4 substrate]                                                                                                                                                              |  | Frequent monitoring for adverse reactions and toxicity related to methadone, including QT prolongation, is recommended. Dose reduction of methadone may be needed.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |  |
| Non-Steroidal Anti- Inflammatory Drugs (NSAIDs)                                                                                                                                                          |  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |  |
| [CYP2C9 substrates]<br>Ibuprofen (400 mg singledose)<br>Diclofenac (50 mg singledose)                                                                                                                    |  | Frequent monitoring for adverse reactions and toxicity related to NSAIDs is recommended. Dose reduction of NSAIDs may be needed.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |  |
| Omeprazole (40 mg QD)* [CYP2C19 inhibitor; CYP2C19and CYP3A4 substrate]                                                                                                                                  |  | No dose adjustment of voriconazole is recommended. When initiating voriconazole in patients alreadyreceiving omeprazole doses of 40 mg or above, it is recommended that the omeprazole dose be halved.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |  |
| Oral Contraceptives* [CYP3A4 substrate; CYP2C19inhibitor]<br>Norethisterone/ethinyloestradiol (1 mg/0.035 mg QD)                                                                                         |  | Monitoring for adverse reactions related to oral contraceptives, in addition to those for voriconazole, is recommended                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |  |
| Short Acting Opiates [CYP3A4 substrates]<br>Alfentanil (20 µg/kg singledose, with concomitant naloxone)<br>Fentanyl (5 µg/kg single dose)                                                                |  | Dose reduction of alfentanil, fentanyl and other short acting opiates similar in structure to alfentanil and metabolised by CYP3A4 (e.g., sufentanil) should be considered.<br>Extended and frequent monitoring for respiratory depression and other opiate associated adverse reactions is recommended.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |  |
| Statins (e.g., lovastatin) [CYP3A4 substrates]                                                                                                                                                           |  | Dose reduction of statins should be considered.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |  |
| Sulfonylureas (e.g., tolbutamide, glipizide, glyburide) [CYP2C9 substrates]                                                                                                                              |  | Careful monitoring of blood glucose is recommended. Dose reduction of sulfonylureas should be considered.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |  |
| Vinca Alkaloids (e.g., vincristine and vinblastine) [CYP3A4 substrates]                                                                                                                                  |  | Dose reduction of vinca alkaloids should be considered.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |  |
| Other HIV Protease Inhibitors (e.g., saquinavir, amprenavir and nelfinavir)* [CYP3A4 substrates and inhibitors]                                                                                          |  | Careful monitoring for any occurrence of drug toxicity and / or lack of efficacy, and dose adjustment may be needed.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |  |
| Other Non-Nucleoside Reverse Transcriptase Inhibitors (NNRTIs) (e.g., delavirdine, nevirapine)* [CYP3A4 substrates, inhibitors or CYP450 inducers]                                                       |  | Careful monitoring for any occurrence of drug toxicity and/or lack of efficacy, and dose adjustment may be needed.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |  |
| Cimetidine (400 mg BID) [non-specific CYP450 inhibitor and increases gastric pH]                                                                                                                         |  | No dose adjustment                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |  |
| Digoxin (0.25 mg QD) [P-gp substrate]                                                                                                                                                                    |  | No dose adjustment                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |  |
| Indinavir (800 mg TID) [CYP3A4 inhibitor and substrate]                                                                                                                                                  |  | No dose adjustment                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |  |
| Macrolide antibiotics<br>Erythromycin (1 g BID) [CYP3A4 inhibitor]<br>Azithromycin (500 mg QD)                                                                                                           |  | No dose adjustment                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |  |
| Mycophenolic acid (1 g single dose) [UDP-glucuronyl transferasesubstrate]                                                                                                                                |  | No dose adjustment                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |  |
| Prednisolone (60 mg single dose) [CYP3A4 substrate]                                                                                                                                                      |  | No dose adjustment                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |  |
| Ranitidine (150 mg BID) [increases gastric pH]                                                                                                                                                           |  | No dose adjustment                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |  |

| System Organ Class                                   |  | Adverse Reactions                                                                                                                                                                                                                                                                                             |  |
|------------------------------------------------------|--|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--|
| Cardiac disorders                                    |  | arrhythmia supraventricular, tachycardia, bradycardia, ventricular fibrillation, ventricular extrasystoles, ventricular tachycardia, electrocardiogram QT prolonged, supraventricular tachycardia, torsades de pointes, atrioventricular block complete, bundle branch block, nodal rhythm                    |  |
| Vascular disorders                                   |  | hypotension, phlebitis, thrombophlebitis, lymphangitis                                                                                                                                                                                                                                                        |  |
| Respiratory, thoracic and mediastinal disorders      |  | respiratory distress <sup>8</sup> , acute respiratory distress syndrome, pulmonary oedema                                                                                                                                                                                                                     |  |
| Gastrointestinal disorders                           |  | Diarrhoea, vomiting, abdominal pain, nausea, cheilitis, dyspepsia, constipation, gingivitis, peritonitis, pancreatitis, swollen tongue, duodenitis, gastroenteritis, glossitis.                                                                                                                               |  |
| Hepatobiliary disorders                              |  | Liver function test abnormal, jaundice, icterus cholestatic, hepatitis <sup>9</sup> , hepatic failure, hepatomegaly, cholecystitis,cholelithiasis.                                                                                                                                                            |  |
| Skin and subcutaneous tissue disorders               |  | rash, dermatitis exfoliative, alopecia, rash maculo-papular, pruritus, erythema, Stevens-Johnson syndrome, phototoxicity, purpura, urticaria, dermatitis allergic, rash papular, rash macular, eczema, toxic epidermal necrolysis, angioedema, pseudoporphyria erythema multiforme, psoriasis, drug eruption. |  |
| Musculoskeletal and connective tissue disorders      |  | back pain, arthritis                                                                                                                                                                                                                                                                                          |  |
| Renal and urinary disorders                          |  | Renal failure acute, haematuria, renal tubular necrosis, proteinuria, nephritis.                                                                                                                                                                                                                              |  |
| General disorders and administration site conditions |  | Pyrexia, chest pain, face oedema <sup>11</sup> , asthenia, chills, infusion site reaction, influenza like illness.                                                                                                                                                                                            |  |
| Investigations                                       |  | Blood creatinine increased, blood urea increased, blood cholesterol increased.                                                                                                                                                                                                                                |  |

<sup>1</sup> Includes febrile neutropenia and neutropenia.  
<sup>2</sup> Includes immune thrombocytopenic purpura.  
<sup>3</sup> Includes nuchal rigidity and tetany.  
<sup>4</sup> Includes hypoxic-ischaemic encephalopathy and metabolic encephalopathy.  
<sup>5</sup> Includes akathisia and parkinsonism.  
<sup>6</sup> See "Visual impairments" paragraph in section 8.  
<sup>8</sup> Includes dyspnoea and dyspnoea exertional.  
<sup>9</sup> Includes drug-induced liver injury, hepatitis toxic, hepatocellular injury and hepatotoxicity.  
<sup>11</sup> Includes periorbital oedema, lip oedema, and oedema mouth

**Description of selected adverse reactions**

*Visual impairment*  
Visual impairments (including blurred vision, photophobia, chloropsia, chromatopsia, colour blindness, cyanopsia, eye disorder, halo vision, night blindness, oscillopsia, photopsia, scintillating scotoma, visual acuity reduced, visual brightness, visual field defect, vitreous floaters, and xanthopsia) with voriconazole were very common. These visual *impairments* were transient and fully reversible, with the majority spontaneously resolving within 60 minutes and no clinically significant long-term visual effects were observed. There was evidence of attenuation with repeated doses of voriconazole. The visual impairment were generally mild, rarely resulted in discontinuation and were not associated with long-term sequelae. Visual impairment may be associated with higher plasma concentrations and/or doses.

The mechanism of action is unknown, although the site of action is most likely to be within the retina.

*Dermatological reactions*  
The majority of rashes were of mild to moderate severity. Patients have developed serious cutaneous reactions, including Stevens-Johnson syndrome (uncommon), toxic epidermal necrolysis (rare) and erythema multiforme (rare) during treatment with voriconazole.

If patients develop a rash they should be monitored closely and Voriconazole discontinued if lesions progress. Photosensitivity reactions such as epheletes, lentigo and actinic keratosis have been reported, especially during long-term therapy.

There have been reports of squamous cell carcinoma of the skin in patients treated with Voriconazole for long periods of time; the mechanism has not been established.

*Liver function tests*  
Liver function test abnormalities may be associated with higher plasma concentrations and/or doses.

The majority of abnormal liver function tests either resolved during treatment without dose adjustment or following dose adjustment, including discontinuation of therapy.

Voriconazole has been associated with cases of serious hepatic toxicity in patients with other serious underlying conditions. This includes cases of jaundice, hepatitis and hepatic failure leading to death.

**13. Overdose & Treatment**

There were cases of accidental overdose which occurred in paediatric patients, who received up to five times the recommended intravenous dose of voriconazole. A single adverse reaction of photophobia of 10 minutes duration was reported.

There is no known antidote to voriconazole.

Voriconazole is haemodialysed with a clearance of 121 mL/min. In an overdose, haemodialysis may assist in the removal of voriconazole from the body.

**14. Ingredients**

**Tablet core Voriconazole**  
Lactose monohydrate,  
Pre gelatinized starch, Croscarmellose sodium, Povidone, Magnesium Stearate.

**Film-coating (50 mg)**  
Hypromellose  
Lactose monohydrate Triacetin, Titanium dioxide

**Film-coating (200 mg)**  
Opadry II White 30K580005

**15. Storage Conditions**

Store below 30°C.

**16. Dosage forms and packaging available**

Voriconazole Tablets 50 mg & 200 mg available in clear PVC-Alu blister pack of 10 Tablets.

**17. Name and address of manufacturer**

**INTAS PHARMACEUTICALS LTD.**  
PLOT NUMBERS 457 458 & 1512/18P  
SARKHE-J BAVLA HIGHWAY MATODA SANAND  
AHMEDABAD GUJARAT IN-382210 INDIA

**18. Product Registration Holder**

**Accord Healthcare Sdn Bhd** (1035160 D)  
Suite 12A-15, Level 12A, Wisma Zelan  
No 1 Jalan Tasik Permaisuri 2  
Bandar Tun Razak  
56000 Kuala Lumpur

**19. Date of revision of PI**

20.04.2026

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INP113

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| <p><b>10. Fertility, Pregnancy and Lactation</b></p> <p><b>Pregnancy</b><br/>There are no adequate data from the use of voriconazole in pregnant women. The potential risk for humans is unknown.<br/>Voriconazole must not be used during pregnancy unless the benefit to the mother clearly outweighs the potential risk to the foetus.</p> <p><b>Women of child-bearing potential</b><br/>Women of child-bearing potential must always use effective contraception during treatment.</p> <p><b>Breast-feeding</b><br/>The excretion of voriconazole into breast milk has not been investigated. Breast-feeding must be stopped on initiation of treatment with Voriconazole.</p>                                                                                                                                                                                                                                                                                                                                                          |  |
| <p><b>11. Effects on ability to drive and use machines</b></p> <p>Voriconazole has moderate influence on the ability to drive and use machines. It may cause transient and reversible changes to vision, including blurring, altered/enhanced visual perception and/or photophobia. Patients must avoid potentially hazardous tasks, such as driving or operating machinery while experiencing these symptoms.</p> <p><b>12. Undesirable effects</b></p> <p><b>Summary of safety profile</b><br/>The most commonly reported adverse reactions were visual impairment, pyrexia, rash, vomiting, nausea, diarrhoea, headache, peripheral oedema, liver function test abnormal, respiratory distress and abdominal pain.</p> <p>The severity of the adverse reactions was generally mild to moderate. No clinically significant differences were seen when the safety data were analysed by age, race, or gender.</p> <p><b>Tabulated list of adverse reactions</b></p> <p>Undesirable effects reported in subjects receiving voriconazole:</p> |  |

Back Side

File Name : RVOR01I3186 -VORITROP(Malaysia)PIL  
Size : 170 x 550 (mm)  
Colour : Pantone Black  
Date : 21\*04\*26